BASELINE → TREATMENT → FOLLOW-UP

Required safety checks and your individual cancer-assessment plan

These stages are not equal time intervals. Your team provides dates and reviews results. There is no universal blood tumour-marker panel or scan interval in this guide.

Check	Before starting	During treatment	Between visits	Response review	After treatment
Bloods and organ safety	Full blood count, kidney and liver tests, glucose and thyroid tests; review autoimmune disease, transplants, previous immune side effects and all medicines.	Full blood count, kidney and liver tests and glucose before every 2-week cycle. Thyroid tests once a month and when indicated; cortisol when clinically indicated.	New symptoms may require extra tests before the next planned review.	Safety bloods do not by themselves show whether treatment controls the cancer.	Keep reporting new symptoms after treatment ends. Immune side effects require monitoring for at least five months after the last dose. Steroid tapers, hormone replacement or specialist review may continue longer. Scans and examinations follow your individual response and follow-up plan.
Thyroid and other hormones	Baseline thyroid function and glucose.	Thyroid function once a month and when clinically indicated.	New fatigue, dizziness, headache or marked thirst needs review.	Cortisol and other tests when clinically indicated.	Continue hormone follow-up and replacement when prescribed.
Head and neck function	Record weight, intake, swallowing, speech and existing mouth or dental problems.	Review pain, intake, weight and support needs.	Report choking, inability to drink, worsening swallowing or new neck symptoms.	Agree examinations and scans with the oncology team.	Continue rehabilitation and symptom support as needed.
Medicines and practical plan	Confirm the exact prescription, treatment aim and actual urgent contacts.	Bring your medicine list, current support plan and treatment record.	Record symptoms and medicines taken; ask before adding a new product.	Confirm any pause, dose change or stop in writing.	Record the final dose and the teams responsible for follow-up.

The team arranges and reviews tests. This guide shows expected timing; it does not record completed appointments. National Cancer Control Programme (NCCP) 00483 v13a: official regimen

Tests in your plan

Full blood count

WHEN

Before starting and before each treatment cycle

Checks blood cells and helps assess infection, anaemia and bleeding risk.

Kidney and liver tests

WHEN

Before starting and before each cycle

Checks for organ injury, including possible immune inflammation.

Blood glucose

WHEN

Baseline and before each cycle

Checks blood sugar and possible treatment-related diabetes.

Thyroid function

WHEN

Baseline; monthly and when indicated

Checks for an underactive or overactive thyroid.

Cortisol

If part of your plan

WHEN

When clinically indicated

Assesses possible adrenal or pituitary problems when symptoms or findings suggest them.

Scans and head and neck examination

If part of your plan

WHEN

As prescribed in the individual disease-assessment plan

Assesses the cancer and the response to treatment separately from safety bloods.

THE WHOLE TREATMENT JOURNEY

Nivolumab every 2 weeks for head and neck cancer: treatment and checks

Recurrent or metastatic squamous cell carcinoma of the head and neck after platinum treatment; the cancer cannot be treated with curative local therapy and has progressed or returned within six months of the last platinum dose.

Nivolumab is given as 240 mg through a vein over 30 minutes OR 600 mg as an injection under the skin over 3–5 minutes, on Day 1 every 2 weeks. Your team selects one formulation; these doses are not interchangeable milligram for milligram. Allow extra time for safety checks, preparation and observation. Treatment continues until the cancer progresses or side effects become unacceptable, with individual benefit and safety review. There is no fixed number of cycles in this head and neck treatment course.

Read from left to right. Columns show stages, not equal lengths of time. This is a general plan: no dates or personal information are needed. Your team confirms which steps apply and arranges appointments.

Treatment, tests and reviews across the course

Care / check	Before starting	During treatment	Between visits	Response review	After treatment
Bloods and organ safety NCCP	Full blood count, kidney and liver tests, glucose and thyroid tests; review autoimmune disease, transplants, previous immune side effects and all medicines.	Full blood count, kidney and liver tests and glucose before every 2-week cycle. Thyroid tests once a month and when indicated; cortisol when clinically indicated.	New symptoms may require extra tests before the next planned review.	Safety bloods do not by themselves show whether treatment controls the cancer.	Keep reporting new symptoms after treatment ends. Immune side effects require monitoring for at least five months after the last dose. Steroid tapers, hormone replacement or specialist review may continue longer. Scans and examinations follow your individual response and follow-up plan.
Thyroid and other hormones NCCP	Baseline thyroid function and glucose.	Thyroid function once a month and when clinically indicated.	New fatigue, dizziness, headache or marked thirst needs review.	Cortisol and other tests when clinically indicated.	Continue hormone follow-up and replacement when prescribed.
Head and neck function NCCP	Record weight, intake, swallowing, speech and existing mouth or dental problems.	Review pain, intake, weight and support needs.	Report choking, inability to drink, worsening swallowing or new neck symptoms.	Agree examinations and scans with the oncology team.	Continue rehabilitation and symptom support as needed.
Medicines and practical plan NCCP	Confirm the exact prescription, treatment aim and actual urgent contacts.	Bring your medicine list, current support plan and treatment record.	Record symptoms and medicines taken; ask before adding a new product.	Confirm any pause, dose change or stop in writing.	Record the final dose and the teams responsible for follow-up.

What each check is for

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Nivolumab monotherapy

Every 2 weeks

Nivolumab aims to control head and neck cancer that has returned or spread, relieve symptoms and help some people live longer. Benefit and side effects are reviewed for you.

Ask / act: Confirm the current phase, prescription, tests and support plan.

Sources: NCCP

At the next review, ask

- Which treatment phase am I in, and what happens next?
- Which test is expected next, who arranges it, and who explains the result?
- Who should I contact if an expected appointment has not arrived?

This map does not show whether a test has been ordered, booked or completed. Do not wait for a scheduled test if you develop new symptoms.

Sources checked 2026-11-09 · Source reconciled draft; independent clinical, pharmacy and patient/PPI review pending · SACTCheck v0.84.46